

Supplemental Material

Cost-Related Barriers to Care and Medications Among US Stroke Survivors, 2019–2025

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Reference numbers refer to the reference list of the main manuscript. Contents: Supplemental Methods, including Tables S1 and S2; Figure S1; Tables S3–S11; STROBE checklist (Table S12).

Supplemental Methods

Data sources and harmonization

Microdata came from the seven annual NHIS Sample Adult files for 2019–2025, the seven annual multiply imputed income files, and the 2020 partial Sample Adult file. Documentation comprised the annual Sample Adult codebooks, survey descriptions, and income imputation technical documents. The download location, size, and SHA-256 hash of each of the 36 files are recorded in the source manifest distributed with the analysis code.

A questionnaire crosswalk records the exact wording, response universe, and codebook page of each stroke and affordability question in every year; a covariate crosswalk does the same for education, insurance, clinical, smoking, and disability variables. Question wording and universes were substantively identical across 2019–2025 after normalizing capitalization in the 2019 prescription-use item. Stroke history was defined as a reported prior clinician diagnosis; neither subtype nor recency was inferred.

Annual estimates retain the full 2020 Sample Adult file, including follow-back respondents, with the annual weight WTFA_A. Pooled estimates retain only the 2020 records that link to the partial file and use the partial-sample weight WTSA_P, matching households on the partial-file identifier. All other years use WTFA_A. Pooled weights were divided by seven so that weighted totals represent an average annual population.

Survey designs were specified on the full adult files using the published stratum (PSTRAT) and primary sampling unit (PPSU) identifiers, which NCHS instructs users to combine across years without modification for 2019 onward. Domain estimation for stroke survivors retained the full design, so variance estimates account for the fact that the stroke domain is a subset of each sampling unit.

Outcome classification

Table S1. Outcome definitions and NHIS source variables

Measure	NHIS variables	Universe
Stroke history	STREV_A	All sample adults
Delayed medical care	MEDDL12M_A	All sample adults

Measure	NHIS variables	Universe
Forgone medical care	MEDNG12M_A	All sample adults
Forgone needed prescriptions	RXDG12M_A	All sample adults
Prescription use	RX12M_A	All sample adults
Dose skipping, dose reduction, delayed filling	RXSK12M_A; RXLS12M_A; RXDL12M_A	RX12M_A=1
Any affordability barrier	Any applicable positive among six items	Known applicability and response rules described below

Response codes 1 and 2 indicate "yes" and "no." Refused, not ascertained, don't know, and missing responses were never recoded as "no." Any applicable "yes" defined a positive composite even if another component was unknown. An all-negative composite required a known response to every applicable item. Prescription nonuse (RX12M_A=2) is an explicit branch that leaves the three medication-use items structurally blank; if prescription use was unknown and all universal items were negative, the primary outcome was classified as unknown. The medication-underuse denominator includes only respondents who reported prescription use and had a classifiable underuse outcome.

Covariate definitions

Table S2. Covariate definitions and NHIS source variables

Domain	Variables	Analysis categories
Age	AGEP_A	18–85; 85 is top-coded; natural spline or 18–64/65+
Sex	SEX_A	Male; female
Race/Hispanic origin	HISPALLP_A	Seven published categories
Region	REGION	Northeast; Midwest; South; West
Education	EDUC_A; EDUCP_A	Four harmonized attainment groups
Income	RATCAT_A; IMPNUM or IMPNUM_A	<100%; 100–199%; 200–399%; ≥400% FPL
Insurance	NOTCOV_A and seven coverage indicators	Private; public without private; military only; uninsured
Clinical history	HYPEV_A; DIBEV_A; CHDEV_A	Yes/no hypertension, diabetes, coronary disease
Smoking	SMKCIGST_A	Never; former; current; indeterminate missing
Self-rated health	PHSTAT_A	Fair/poor vs good/very good/excellent
Disability	DISAB3_A	Published Washington Group disability recode

For the individual coverage indicators, codes 1 and 2 both indicate coverage and code 3 indicates no coverage; the uninsured variable NOTCOV_A codes 1 as not covered and 2 as covered. The insurance hierarchy assigned private coverage first among covered respondents, then public coverage without private coverage, then military coverage only when every other listed coverage type was explicitly absent. Ambiguous coverage patterns were set to missing. Private coverage can coexist with public or military coverage, and public-without-private coverage can coexist with military coverage.

Income used the categorical poverty-ratio variable RATCAT_A rather than the rounded continuous ratio. Categories 1–3 correspond to below 100% of the federal poverty level (FPL), 4–7 to 100%–199%, 8–11 to 200%–399%, and 12–14 to 400% or higher. The imputation index is named IMPNUM in 2019–2020 and IMPNUM_A thereafter. All ten annual imputations were linked by year, household, and index, with assertions of unique keys and complete matching. In

descriptive tables the income point estimate is the mean of the ten completed-data weighted percentages, and unweighted income counts are averages over imputations.

Estimands and models

The primary descriptive estimand is the survey-weighted prevalence of any cost-related barrier among noninstitutionalized adults reporting prior stroke with a classifiable outcome. Adjusted prevalence ratios condition on the stated covariate set. Model 1 adjusts for categorical year, an age spline, sex, race and Hispanic origin, and region. Model 2 adds education, income, and insurance. Model 3 adds hypertension, diabetes, coronary heart disease, smoking, self-rated health, and disability. All three models use the same 6,777 records. The age-group model sequence substitutes a working-age indicator for the age spline.

For each model, coefficients were averaged over the ten imputations. The total covariance equals the mean within-imputation survey covariance plus 1.1 times the between-imputation covariance of the coefficients. Confidence intervals use the finite-sample degrees of freedom returned by `mitools::MIcombine`, supplied with the minimum residual design degrees of freedom across the ten fits. Poisson coefficients were exponentiated to obtain prevalence ratios. This procedure propagates income-imputation uncertainty.

For standardized annual prevalence, the fitted logistic model assigned each survivor to each survey year in turn while holding other covariates fixed, and the predicted probabilities were averaged with the pooled complete-case weights. The linearized influence function includes the regression-coefficient influence multiplied by the prediction gradient and the weighted deviation of each predicted probability from its standardized mean, so the covariance incorporates uncertainty in the reference distribution and its covariance with model estimation. Imputation-specific margins and their covariance were pooled on the logit scale; the 2025-minus-2019 absolute difference was pooled on the probability scale.

The bounded logistic sensitivity applied the same standardization to income, insurance, disability, and the age-group model, assigning the full analytic sample to the comparison and reference categories in turn and forming their prevalence ratio. These are marginal estimands, distinct from the conditional modified Poisson prevalence ratios.

The categorical-year and year-by-age-group tests are pooled Wald F statistics based on the selected coefficients and their Rubin total covariance, with denominator degrees of freedom equal to the minimum scalar pooled degrees of freedom among those coefficients. Missing-information fractions for the year terms were small.

Confidence intervals and precision screen

Annual observed confidence intervals are design-based logit intervals. For pooled component estimates, an effective sample size was calculated from the prevalence and its Taylor variance, capped at the observed sample size and adjusted by the squared ratio of the 0.975 normal quantile to the corresponding design-based t quantile; beta quantiles based on that effective

sample size provide bounded intervals. Subgroup intervals were pooled across imputations on the logit scale.

Following NCHS data presentation standards,[11] a proportion was flagged when its unweighted or effective sample size was below 30, its confidence-interval width exceeded 0.30, or its width exceeded 0.05 and was more than 130% of the smaller of the estimate and its complement; design degrees of freedom below eight prompted review. All main annual, component, and reported subgroup estimates passed. The unknown-insurance subgroup (29 records) failed the screen and is not reported.

Sensitivity analyses and verification

Component-outcome models used the primary-model complete-covariate population restricted to observed values of the component, so their denominators differ from the broader component prevalence denominators. The no-disability model omits the disability term. The working-age model retains a continuous age spline within ages 18–64 years. The no-2020 model removes 2020 records and the corresponding year level. The missing-category analysis retains all 7,104 observed primary outcomes and includes explicit unknown levels for incomplete nonincome covariates.

All 160 modified Poisson fits and all 20 bounded logistic fits converged. The fully adjusted modified Poisson model produced 14–16 fitted values above one per imputation, with a maximum of approximately 1.9. Independent verification in Python reconstructed 12 annual and pooled primary estimates and their Taylor standard errors directly from the source files, reproduced the first-imputation fully adjusted model from the exported design matrix (maximum absolute coefficient discrepancy below 3×10^{-13} ; maximum covariance discrepancy below 3×10^{-9}), and checked Rubin-pooled means and standard errors for all 16 model specifications.

Analytic decisions

Employment status was not included because it overlaps strongly with age and disability and would have required additional harmonization across years. A year-by-insurance interaction was not fit because the military-only group contributed only 109 observed records and 17 events across the seven years. No data-driven significance criterion was used to select the final model. The ten records with unknown age were excluded rather than imputed.

Supplemental Figure

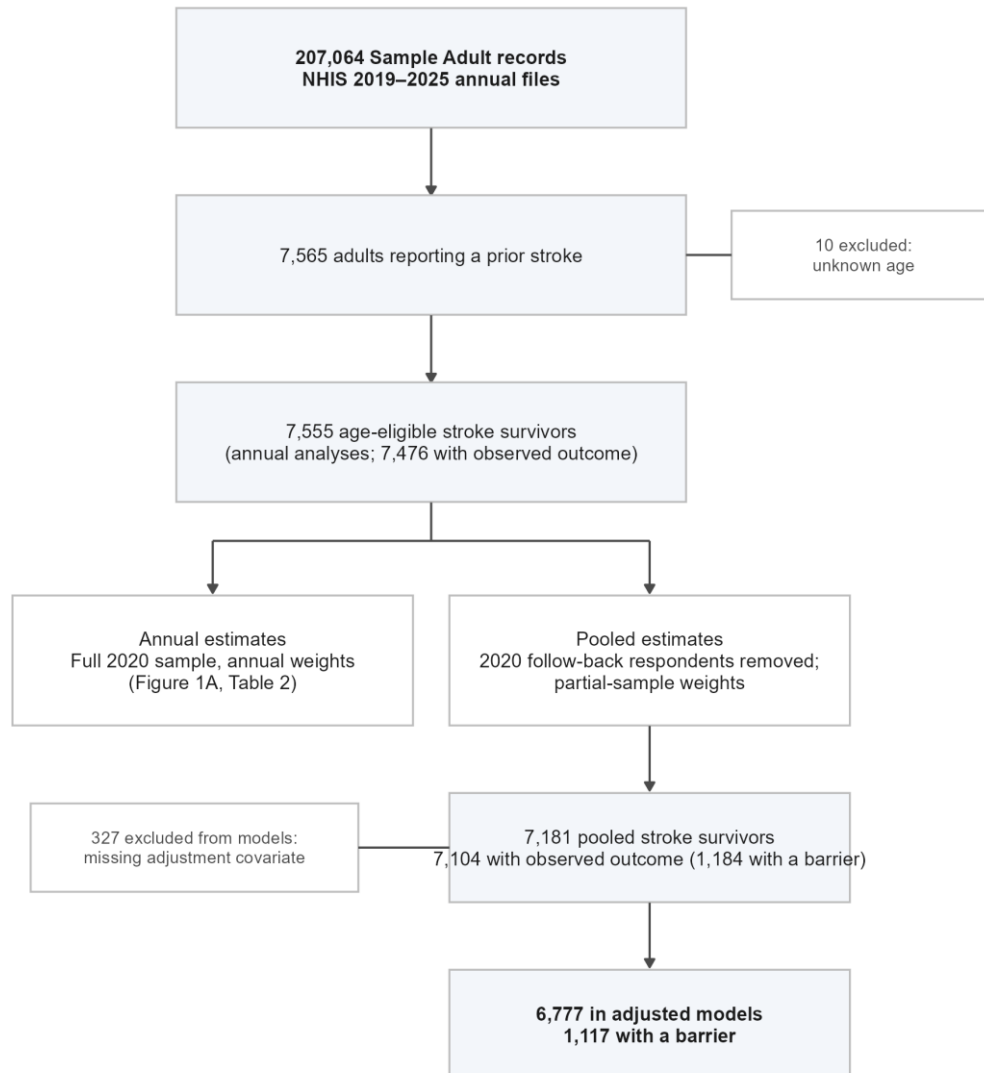


Figure S1. Selection of the annual and pooled analytic samples. Annual and pooled samples apply different inclusion and weighting rules for 2020. The 7,555 annual stroke records include the full 2020 sample; the 7,181 pooled stroke records exclude the 2020 follow-back respondents and use the partial-sample weights. Adjusted models additionally require an observed outcome and complete nonincome covariates.

Supplemental Tables

Table S3. Annual and pooled sample counts by survey year

Year	Annual stroke survivors	Annual, outcome observed	Pooled stroke survivors	Pooled, outcome observed	Pooled, with barrier
2019	1,201	1,191	1,201	1,191	231
2020	1,059	1,051	685	679	105
2021	1,010	1,001	1,010	1,001	179
2022	1,008	999	1,008	999	147
2023	1,131	1,116	1,131	1,116	169
2024	1,244	1,229	1,244	1,229	215
2025	902	889	902	889	138

Annual stroke records total 7,555; pooled stroke records total 7,181. The difference comes from excluding 2020 followback records in pooled analyses. These are unweighted counts.

Table S4. Missing outcome and nonincome covariate values in the pooled stroke-survivor sample

Variable	No. missing	Missing, %
Primary outcome (any cost-related barrier)	77	1.07
Age	0	0.00
Sex	0	0.00
Race and Hispanic origin	0	0.00
Region	0	0.00
Education	35	0.49
Insurance	34	0.47
Hypertension	9	0.13
Diabetes	10	0.14
Coronary heart disease	68	0.95
Self-rated health	11	0.15
Smoking	253	3.52
Disability	0	0.00

Denominator is 7,181 stroke records. Missingness overlaps across variables. Among records with observed primary outcomes, 327 had at least one missing adjustment variable.

Table S5. Complete characteristics of the pooled sample, overall and by cost-related barrier status

Characteristic	Overall (N=7,104)	No barrier (n=5,920)	Any barrier (n=1,184)
Age 65 years or older	4,763 (59.0)	4,237 (64.0)	526 (35.9)
Age 18–64 years	2,341 (41.0)	1,683 (36.0)	658 (64.1)
Male	3,269 (48.5)	2,777 (49.7)	492 (43.3)
Female	3,835 (51.5)	3,143 (50.3)	692 (56.7)
Hispanic, any race	599 (11.5)	478 (11.2)	121 (12.8)

Characteristic	Overall (N=7,104)	No barrier (n=5,920)	Any barrier (n=1,184)
Non-Hispanic White only	5,052 (66.6)	4,279 (67.6)	773 (61.7)
Non-Hispanic Black only	1,068 (15.8)	852 (14.9)	216 (19.7)
Non-Hispanic Asian only	171 (3.3)	146 (3.5)	25 (2.4)
Non-Hispanic AIAN only	77 (0.9)	66 (0.9)	11 (0.8)
Non-Hispanic AIAN with another race	91 (1.3)	63 (1.1)	28 (1.8)
Other single or multiple races	46 (0.7)	36 (0.6)	10 (0.8)
Northeast	1,032 (15.6)	901 (16.4)	131 (11.8)
Midwest	1,681 (22.0)	1,405 (22.3)	276 (20.6)
South	2,942 (43.1)	2,362 (41.0)	580 (52.7)
West	1,449 (19.4)	1,252 (20.3)	197 (15.0)
Bachelor's degree or higher	1,670 (20.1)	1,437 (20.6)	233 (17.4)
Some college or associate	2,149 (28.7)	1,743 (28.2)	406 (30.9)
High school or GED	2,202 (32.3)	1,848 (32.6)	354 (30.8)
Less than high school	1,050 (18.3)	866 (18.0)	184 (19.7)
Education unknown	33 (0.6)	26 (0.5)	7 (1.2)
Income $\geq$ 400% FPL	1,681.2 (24.2)	1,559.9 (27.0)	121.3 (11.0)
Income 200–399% FPL	2,133.3 (31.0)	1,786.9 (31.0)	346.4 (31.2)
Income 100–199% FPL	1,986.9 (27.5)	1,549.6 (25.6)	437.3 (36.0)
Income <100% FPL	1,302.6 (17.4)	1,023.6 (16.4)	279.0 (21.8)
Private, with or without public coverage	2,888 (40.3)	2,465 (40.9)	423 (37.2)
Public without private	3,871 (53.6)	3,263 (54.9)	608 (47.4)
Military only	109 (1.6)	92 (1.6)	17 (1.7)
Uninsured	207 (4.0)	80 (2.0)	127 (13.2)
Insurance unknown	29 (0.5)	20 (0.5)	9 (0.6)
No hypertension	1,747 (25.7)	1,455 (25.3)	292 (27.8)
Hypertension	5,349 (74.2)	4,458 (74.6)	891 (72.1)
Hypertension unknown	8 (0.1)	7 (0.1)	1 (0.1)
No diabetes	5,136 (71.3)	4,330 (72.3)	806 (66.9)
Diabetes	1,960 (28.6)	1,583 (27.6)	377 (33.0)
Diabetes unknown	8 (0.1)	7 (0.1)	1 (0.0)
No coronary heart disease	5,212 (74.0)	4,369 (74.8)	843 (70.8)
Coronary heart disease	1,828 (25.2)	1,500 (24.5)	328 (28.4)
Coronary heart disease unknown	64 (0.7)	51 (0.7)	13 (0.8)
Never smoked	3,196 (46.0)	2,710 (46.6)	486 (43.5)
Former smoker	2,592 (34.0)	2,240 (35.5)	352 (27.2)
Current smoker	1,132 (16.8)	821 (14.7)	311 (26.3)
Smoking unknown	184 (3.2)	149 (3.2)	35 (3.0)
Good or better self-rated health	3,607 (48.0)	3,201 (51.6)	406 (31.7)
Fair or poor self-rated health	3,486 (51.9)	2,712 (48.3)	774 (68.1)
Self-rated health unknown	11 (0.1)	7 (0.1)	4 (0.2)
Without disability	4,384 (61.2)	3,742 (62.3)	642 (56.3)
With disability	2,720 (38.8)	2,178 (37.7)	542 (43.7)

Values are unweighted number (survey-weighted column percentage). Income counts are averaged across the ten NCHS imputations and may be fractional. Unknown values are retained in denominators; percentages within a variable may not sum to 100 because of rounding. FPL indicates federal poverty level; NHIS, National Health Interview Survey. AIAN, American Indian or Alaska Native. The race/Hispanic-origin categories follow the published public-use recode.

Table S6. Pooled prevalence of the composite outcome and each component barrier, 2019–2025

Outcome	No. with observed outcome	No. with barrier	Prevalence, % (95% CI)
Any cost-related barrier	7,104	1,184	17.9 (16.8–19.1)
Universal three-question composite	7,106	1,012	15.5 (14.3–16.6)
Delayed medical care	7,116	554	8.5 (7.7–9.4)
Forgone medical care	7,115	541	8.4 (7.6–9.3)
Forgone needed prescriptions	7,103	647	10.0 (9.1–11.0)
Medication underuse among users	6,754	635	10.1 (9.2–11.1)

Components overlap. Medication-underuse estimates use prescription users only. Each prevalence denominator includes all stroke records with a classifiable corresponding outcome; it is not restricted to complete model covariates. All displayed estimates pass the implemented precision screen.

Table S7. Unadjusted pooled prevalence of any cost-related barrier in principal subgroups

Subgroup	No. with observed outcome	Prevalence, % (95% CI)
Age 65 years or older	4,763	10.9 (9.8–12.0)
Age 18–64 years	2,341	28.0 (25.9–30.2)
Income ≥400% FPL	1,681.2	8.2 (6.7–10.0)
Income 200–399% FPL	2,133.3	18.0 (16.0–20.3)
Income 100–199% FPL	1,986.9	23.5 (21.1–26.1)
Income <100% FPL	1,302.6	22.5 (19.6–25.6)
Private, with or without public coverage	2,888	16.6 (14.9–18.4)
Public without private	3,871	15.8 (14.5–17.3)
Military only	109	18.2 (11.1–28.4)
Uninsured	207	58.7 (50.7–66.3)
Without disability	4,384	16.5 (15.1–17.9)
With disability	2,720	20.2 (18.4–22.1)

Income counts are averages across ten imputations. The unknown-insurance subgroup (29 records) failed the precision screen, and its prevalence is not displayed. These are unadjusted subgroup estimates.

Table S8. Fully adjusted (Model 3) prevalence ratios for all covariates

Characteristic	PR (95% CI)	P value
Year 2020 vs 2019	0.89 (0.69–1.14)	0.351
Year 2021 vs 2019	0.96 (0.78–1.19)	0.702
Year 2022 vs 2019	0.84 (0.67–1.04)	0.112
Year 2023 vs 2019	0.85 (0.69–1.05)	0.140
Year 2024 vs 2019	1.06 (0.87–1.29)	0.569
Year 2025 vs 2019	0.89 (0.71–1.10)	0.274
Female vs male	1.27 (1.12–1.43)	<0.001
Non-Hispanic White only vs Hispanic	1.20 (0.95–1.51)	0.124

Characteristic	PR (95% CI)	P value
Non-Hispanic Black only vs Hispanic	1.14 (0.88–1.48)	0.323
Non-Hispanic Asian only vs Hispanic	0.78 (0.45–1.35)	0.367
Non-Hispanic AIAN only vs Hispanic	0.82 (0.42–1.61)	0.570
Non-Hispanic AIAN with another race vs Hispanic	1.06 (0.69–1.63)	0.800
Other single or multiple races vs Hispanic	1.26 (0.71–2.25)	0.431
Midwest vs Northeast	1.21 (0.97–1.50)	0.098
South vs Northeast	1.42 (1.15–1.75)	<0.001
West vs Northeast	1.24 (0.96–1.61)	0.095
Some college/associate vs bachelor's or higher	0.89 (0.75–1.06)	0.202
High school/GED vs bachelor's or higher	0.75 (0.62–0.89)	0.001
Less than high school vs bachelor's or higher	0.80 (0.64–0.99)	0.038
Income 200–399% vs ≥400% FPL	2.07 (1.65–2.61)	<0.001
Income 100–199% vs ≥400% FPL	2.50 (1.94–3.21)	<0.001
Income <100% vs ≥400% FPL	2.00 (1.52–2.63)	<0.001
Public without private vs private	0.72 (0.63–0.83)	<0.001
Military only vs private	0.81 (0.52–1.28)	0.372
Uninsured vs private	1.79 (1.49–2.16)	<0.001
Hypertension vs none	0.99 (0.86–1.14)	0.863
Diabetes vs none	1.12 (0.97–1.30)	0.126
Coronary heart disease vs none	1.37 (1.18–1.58)	<0.001
Former vs never smoking	0.98 (0.85–1.14)	0.820
Current vs never smoking	1.21 (1.04–1.42)	0.015
Fair/poor vs good or better health	1.58 (1.37–1.82)	<0.001
Disability vs none	1.19 (1.05–1.35)	0.006

The model includes an age spline with 3 degrees of freedom. Spline basis coefficients and the intercept are provided in the public repository. No multiplicity correction was applied.

Table S9. Selected prevalence ratios in sensitivity analyses

Analysis	No.	Income 100–199% vs ≥400% FPL	Uninsured vs private	Disability vs none
Exclude 2020	6,124	2.59 (2.00–3.36)	1.82 (1.49–2.21)	1.18 (1.04–1.34)
Age 18–64 only	2,231	2.33 (1.67–3.25)	1.85 (1.51–2.27)	1.08 (0.92–1.27)
No disability	4,209	2.50 (1.79–3.47)	1.80 (1.42–2.28)	Not applicable
Universal composite	6,776	2.45 (1.87–3.22)	2.09 (1.70–2.57)	1.26 (1.10–1.45)
Delayed medical care	6,774	2.58 (1.76–3.79)	2.69 (1.98–3.66)	1.35 (1.10–1.65)
Forgone medical care	6,776	2.46 (1.66–3.65)	2.99 (2.23–4.00)	1.40 (1.15–1.71)
Forgone prescriptions	6,775	2.52 (1.75–3.65)	2.09 (1.58–2.77)	1.24 (1.04–1.49)
Medication underuse	6,449	2.85 (2.01–4.03)	1.74 (1.32–2.31)	1.23 (1.03–1.48)
Unknown-category diagnostic	7,104	2.45 (1.91–3.14)	1.85 (1.55–2.21)	1.16 (1.03–1.31)

Values are PRs (95% CIs) from the corresponding fully adjusted model. No-disability models omit the disability term. The complete coefficient sets, including all income categories, are provided in the public repository. Outcome-specific model denominators differ from Table S6 because they require complete adjustment variables.

Table S10. Standardized prevalence ratios from bounded logistic models

Contrast	Marginal PR (95% CI)	Reference %	Comparison %
Income 200–399% FPL vs Income $\geq$ 400% FPL	1.99 (1.59–2.49)	9.6	19.0
Income 100–199% FPL vs Income $\geq$ 400% FPL	2.41 (1.88–3.09)	9.6	23.0
Income <100% FPL vs Income $\geq$ 400% FPL	1.91 (1.45–2.51)	9.6	18.3
Public without private vs Private, with or without public coverage	0.72 (0.63–0.83)	20.1	14.5
Military only vs Private, with or without public coverage	0.83 (0.52–1.32)	20.1	16.7
Uninsured vs Private, with or without public coverage	1.99 (1.63–2.43)	20.1	40.0
With disability vs Without disability	1.18 (1.04–1.33)	16.7	19.6
Age 18–64 years vs Age 65 years or older	2.13 (1.86–2.44)	12.0	25.5

Logistic predictions remain between zero and one. Marginal PRs are pooled on the log scale; displayed prevalences are arithmetic means of imputation-specific margins, so their displayed ratio may differ slightly from the pooled PR. The age contrast uses the age-group model.

Table S11. Convergence and fitted-value diagnostics for the modified Poisson models

Model	No.	No. with barrier	Fitted values >1 per imputation	Largest fitted value	Convergence
Model 1	6,777	1,117	0–0	0.517	All converged
Model 2	6,777	1,117	3–4	1.190	All converged
Model 3	6,777	1,117	14–16	1.943	All converged
Age-group Model 1	6,777	1,117	0–0	0.466	All converged
Age-group Model 2	6,777	1,117	3–4	1.106	All converged
Age-group Model 3	6,777	1,117	15–17	1.912	All converged
Year by age-group interaction	6,777	1,117	17–19	1.888	All converged
Exclude 2020	6,124	1,016	12–16	1.943	All converged
Age 18–64 only	2,231	620	14–15	2.127	All converged
No disability	4,209	607	5–6	1.802	All converged
Universal composite	6,776	955	13–17	1.972	All converged
Delayed medical care	6,774	523	3–5	1.518	All converged
Forgone medical care	6,776	509	4–8	1.651	All converged
Forgone prescriptions	6,775	610	4–6	1.488	All converged
Medication underuse	6,449	598	5–6	1.709	All converged
Unknown-category diagnostic	7,104	1,184	16–19	1.891	All converged

Ten fits per specification (one per income imputation). The bounded logistic models (Table S10) give similar associations. Age-group models replace the age spline with an age-group indicator.

STROBE Checklist

Table S12. STROBE checklist for cross-sectional studies

Item	Location in manuscript	Content
1	Title and Abstract	Design, population, period and principal quantitative findings.
2–3	Introduction	Background, prior studies and objectives stated.
4–5	Methods / Study design	Repeated cross-sectional design, public NHIS setting and 2019–2025 period.
6	Methods / Population; Figure S1	Eligibility, stroke definition, age exclusion, annual vs pooled selection.
7–8	Methods / Outcomes and covariates; Tables S1–S2	Definitions, universes and cross-year source verification; crosswalk files included.
9	Discussion / Limitations	Self-report, nonresponse, institutional exclusion, confounding and missingness addressed.
10	Methods / Population	All eligible records in the selected years; no formal power-based sample selection.
11	Methods / Covariates	Age spline and categorical income/education definitions given.
12	Methods / Statistical analysis; Supplemental Methods	Survey weighting, MI, missingness, interaction and sensitivity procedures.
13	Results / Study population; Table S3; Figure S1	Counts and reasons for exclusions; different annual and pooled paths.
14	Table 1; Tables S4–S5	Characteristics and missingness; income counts averaged across imputations.
15	Table 2; Tables S6–S7	Outcome counts and annual, component and subgroup estimates.
16	Table 3; Table S8; Results	Adjusted PRs, covariate sets, precision and absolute annual difference.
17	Results / Sensitivity analyses; Tables S9–S11	Sensitivity estimates and model diagnostics.
18–21	Discussion and Conclusions	Key results, limitations, cautious interpretation and generalizability.
22	Declarations	No external funding is reported.